

# Health assessment report

## Claimant details

**Name** Michael Thompson

**National insurance number** QQ123456A

**Date of birth** 14 February 1968

## Assessment details

**Date report was completed** 11 September 2023

**Benefit** Personal Independence Payment

**Assessment type** Telephone Assessment

**Healthcare professional** Alex Huges

## Evidence

**Evidence used**

## Consultation

**Consultation date** 11 September 2023

**Was the claimants identity verified?** Yes

**Did the claimant give consent?** Yes

# Condition History

## Osteoarthritis

### When did the condition or disability start?

Four years ago, by consultant.

### Condition history

Exacerbations/Fluctuations: Pain is constant to knees and does not fluctuate. Shoulder pain has improved since steroid injection, does not get pain to fingers but finds they get stiff, particularly in the morning.

Treatments/Interventions (and effect, including past or planned): Sees consultant once per year, has seen consultant twice in past year for steroid injections to shoulders and knees, consultant also prescribed naproxen at this appointment. Had injection to right shoulder one year ago which has improved condition. However, injection to both knees have not helped and still has bilateral knee pain. Sees GP regularly, however no other planned treatment or injections. Next consultant review in six months' time with no planned treatment expected.

## Back problem

### When did the condition or disability start?

Ten years ago by GP.

### Condition history

Symptoms: Pain to lower back.

Exacerbations/Fluctuations: Pain is a constant dull ache which then becomes a stabbing/shooting pain when he bends and lifts.

Treatments/Interventions (and effect, including past or planned): Had physiotherapy five years ago which slightly improved condition. Now is managed by the GP as required.

## **Depression**

### **When did the condition or disability start?**

Three years ago by GP.

### **Condition history**

Feels low in mood and gets down about the pain.

Exacerbations/Fluctuations: Occurs once per month lasting two-three days, finds coping mechanisms such as spending time with his family.

Treatments/Interventions (and effect, including past or planned): Sees GP for a yearly check-up.

Has never had any thoughts of self-harm or suicide.

## **Heart attack**

### **When did the condition or disability start?**

Diagnosed: seven years ago by consultant.

### **Condition history**

Symptoms: Chest pain when he walks fast or walks up a hill.

Exacerbations/Fluctuations: Chest pain occurs once per month lasting a few seconds per time and GTN spray improves condition.

Treatments/Interventions (and effect, including past or planned): Was admitted to hospital for one week, seven years ago after heart attack, however since then no other treatment and is managed by GP.

No other medical problems reported during the assessment.

None.

No other medical conditions reported at assessment.

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## Medication

### GTN spray

Dosage: As required

Frequency: used about once per month

Reason for taking: to prevent chest pain/angina

Effective

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### Isosorbide

Dosage: 60 mg

Frequency: Once per day

Reason for taking: following heart attack/angina

Effective

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### Co-codamol

Dosage: 30/500 mg

Frequency: two tablets four times daily

Reason for taking: Pain relief

Partially effective

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## **Naproxen**

Dosage: 500 mg

Frequency: twice daily

Reason for taking: Pain relief

Partially effective

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## **Lansoprazole**

Dosage: 15 mg

Frequency: Daily

Reason for taking: To protect stomach

Effective

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## **Sertraline**

Dosage: 50 mg

Frequency: Daily

Reason for taking: For depression

Effective

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## **Atorvastatin**

Dosage: 20 mg

Frequency: Once daily

Reason for taking: (taken following heart attack) preventative measures

Claimant not sure

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## **Aspirin**

Dosage: 75 mg

Frequency: Once daily

Reason for taking: (taken following heart attack) preventative measures

Claimant not sure

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No side effects reported.

No home therapies.

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## **Social and Occupational History**

Lives with partner in a bungalow.

Moved from a house a few years ago, as was struggling with stairs due chest and knee pains.

Shower is over bath; struggling to get in and out and is thinking of buying a rail to support with this.

Has easy grip knives, a rubber bathmat and walking stick.

Stopped working after heart attack. Partner works part time.

Drives automatic car, swapped from manual to automatic two years ago due to knee pain. No adaptations. Is able to get in and out and but will hold on to the door due to knee pain. When driving long distances finds pain increases to back and knees. Can grip steering wheel with

no restrictions. Drives 15-20 minutes maximum as then finds the pain in knees and back increases, can drive these distances several times per week.

Has two adult children who they enjoy seeing weekly, is able to drive there.

Enjoys spending time with grandchildren and will look after them whilst their parents are working, will take them to the park, which is across the road, will need to sit on the bench after walking to the park due to back and knee pain and is then able to walk back.

Will talk to other people they know from the area whilst children play and enjoys this.

Used to enjoy doing leisure activities however has stopped doing these due to struggling to stand because of back and knee pain. Misses doing these hobbies.

Enjoys reading and will do this daily, able to turn pages of a book. Enjoys listening to radio and watching TV. Able to grip and use mobile phone to browse the internet and use social media.

Enjoys cleaning and pottering around the house slowly as and when back and knee pain allows.

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## Daily Living

### Activity 1 – Preparing Food

Can make breakfast and snacks. Partner will prepare dinner as they have always done this, however, enjoys helping peel and chop vegetables, will do this sitting down due to pain in back and knees. Bought an easy grip knife to help peel and chop vegetables as finds this easier. Will help prepare food majority of days.

## **Activity 2 – Taking Nutrition**

Has meals three times per day. Uses standard cutlery. Does not need help with cutting cooked food.

## **Activity 3 – Managing Therapy or Monitoring a Health Condition**

Partner will take medication out the packaging as they have stiff fingers in the morning, however, would be able to do this unaided if partner was not there.

## **Activity 4 – Washing and Bathing**

Is able to get in and out of the bath and shower, however, will lean on the wall for support and does this slowly due to knee and back pain. Has a bathmat to prevent slips/falls and is thinking of getting a grab rail to make it easier. Can wash full body and will sit on the side of the bath to wash feet and lower body as bending causes back pain.

## **Activity 5 – Managing Toilet Needs or Incontinence**

Will hold onto the sink and wall for support when getting on and off the toilet due to pain in back and knees. Is able to clean self afterwards. No continence problems.

## **Activity 6 – Dressing and Undressing**

Able to dress upper and lower body, prefers to wear clothes without fastenings as finds this easier due to pain in hands. Struggles to bend, is able to bend just below knees and then cannot bend anymore to put on socks and lace up shoes, due to back pain and struggles to lift up feet due to knee and back pain, so uses slip on shoes and sits down to dress.

## **Activity 7 – Communicating Verbally**

No restrictions reported. Communicates with health professionals about their condition, understands information being communicated.

## **Activity 8 – Reading and Understanding Signs, Symbols and Words**

No restrictions reported, will read books daily and enjoys doing this.

## **Activity 9 – Engaging with Other People Face to Face**

He is able to engage with family, friends and health professionals as required and enjoys speaking to family and friends and does this regularly. He has no problems talking with unfamiliar people as needed when at appointments or shops.

## **Activity 10 – Making Budgeting Decisions**

Partner manages all the bills as they have always done this. Will look online and do online banking and understands money.

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# **Mobility**

## **Activity 11 – Planning and Following Journeys**

He is able to go out as required to familiar and unfamiliar places and will take grandchildren to park. Can plan and follow a journey without support.

## **Activity 12 – Moving Around**

He is able to walk to the park just across the road (with grandchildren), will sit down (whilst they play) and is able to walk back, does this twice per week weather depending, will use a walking stick when walking and walks at a slightly slower pace than partner, takes approximately 1 minute to get there and takes the same amount of time to walk back. Unsure of how far this is. If they were to walk this distance again it would be at a very slow pace and would struggle with the pain and would find the next day they would have increased pain. However, would still be able to potter around the house the following day.

If needs to go to the shop will go to the small local one as is unable to walk around a big supermarket due to knee and back pain. Parks in a disabled space and is then able to walk to the shop and around the small shop, using a walking stick, they can walk about two aisles at a time, approximately 30-40 metres in total, then will need to rest for approximately one minute before continuing. Would be able to continue doing this in the small shop until task is

completed. They can do this journey once per day, however, would struggle with pain if had to complete this journey more and would find his pace would become slower each time due to increased knee pain.

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## **Observations**

### **Mental State Observations**

Good cognition and insight. Speaks in normal tone, pace, volume of voice sounds jovial.

### **Informal Observations**

Did not sound tired or breathless during the telephone assessment.